

Alcohol Use Disorder Intake Form

Telemedicine Asynchronous Practice | Naltrexone 50mg Treatment Program

Confidential Patient Intake | For Provider Review

Patient Name: _____
Date Completed: _____

Date of Birth: _____
State of Residence: _____

SECTION 1 — DSM-5 Alcohol Use Disorder Screening (Past 12 Months)

Answer Yes or No for each question based on your experience in the past 12 months.

1. **Have you had times when you drank more, or longer, than you intended?**
☐ Yes ☐ No
2. **Have you wanted to cut down or stop drinking but found you couldn't?**
☐ Yes ☐ No
3. **Do you spend a lot of time drinking or recovering from drinking?**
☐ Yes ☐ No
4. **Have you felt a strong urge or craving to drink?**
☐ Yes ☐ No
5. **Has drinking interfered with work, school, or family responsibilities?**
☐ Yes ☐ No
6. **Do you continue to drink even though it causes problems with family or friends?**
☐ Yes ☐ No
7. **Have you given up or reduced activities you enjoy because of drinking?**
☐ Yes ☐ No
8. **Have you continued to drink in physically dangerous situations (e.g., driving)?**
☐ Yes ☐ No
9. **Do you continue to drink even though it causes or worsens depression, anxiety, or other health problems?**
☐ Yes ☐ No
10. **Do you need to drink more than before to get the same effect (tolerance)?**
☐ Yes ☐ No
11. **Have you experienced withdrawal symptoms when you stopped or cut back (sweating, shaking, nausea, anxiety, or seizures)?**
☐ Yes ☐ No

PROVIDER SCORING: Count total Yes answers. 2-3 = Mild AUD | 4-5 = Moderate AUD | 6+ = Severe AUD. Naltrexone is appropriate for moderate-to-severe AUD and may be considered for mild AUD with clinical judgment. Minimum of 2 DSM-5 criteria required for diagnosis.

SECTION 2 — Current Drinking Pattern

12. **How many days per week do you typically drink?**
☐ 1-2 days
☐ 3-4 days
☐ 5-6 days
☐ Every day
13. **On a typical drinking day, how many standard drinks do you have?**
☐ 1-2 drinks
☐ 3-4 drinks
☐ 5-6 drinks
☐ 7 or more drinks
14. **When did you last have a drink?**
☐ Today
☐ Yesterday
☐ 2-7 days ago
☐ More than 1 week ago
15. **In the past month, have you had 5 or more drinks in a day (men) or 4 or more drinks in a day (women)?**

☐ Yes ☐ No

SECTION 3 — Withdrawal Risk Assessment

16. Have you ever had a seizure related to alcohol withdrawal?

☐ Yes ☐ No

17. Have you ever had delirium tremens (DTs) -- severe shaking, confusion, or hallucinations when stopping alcohol?

☐ Yes ☐ No

18. Have you ever been hospitalized specifically for alcohol detox or withdrawal?

☐ Yes ☐ No

19. Do you drink first thing in the morning to avoid feeling shaky, anxious, or sick?

☐ Yes ☐ No

PROVIDER FLAG: Any Yes in Section 3 indicates elevated withdrawal risk. Consider CIWA protocol, in-person evaluation, or supervised medical detox before initiating naltrexone. Do not start naltrexone in patients at high risk for acute withdrawal without appropriate management.

SECTION 4 — Naltrexone Safety Screening (Contraindications)

20. Are you currently using, or have you used in the past 7-10 days, any of the following? (Select all that apply)

- ☐ Prescription opioids (e.g., oxycodone, hydrocodone, codeine, morphine)
- ☐ Buprenorphine / Suboxone / Methadone
- ☐ Heroin
- ☐ Fentanyl or suspected fentanyl (street drugs, counterfeit pressed pills)
- ☐ None of the above

21. Are you currently enrolled in a methadone or buprenorphine maintenance program?

☐ Yes ☐ No

PROVIDER FLAG: Yes to Q20 or Q21 is an ABSOLUTE CONTRAINDICATION. Do not prescribe naltrexone until the patient has been opioid-free for a minimum of 7-10 days (longer for methadone). Risk of precipitating severe acute opioid withdrawal.

22. Have you ever been diagnosed with any of the following liver conditions? (Select one)

- ☐ Cirrhosis (advanced liver scarring)
- ☐ Acute hepatitis (sudden liver inflammation)
- ☐ Liver failure
- ☐ Elevated liver enzymes only -- no formal liver diagnosis
- ☐ None of the above / Unknown

23. Have you had liver blood tests (LFTs) done in the past 6 months?

- ☐ Yes -- results were normal
- ☐ Yes -- results were mildly elevated
- ☐ Yes -- results were significantly elevated
- ☐ No / Unknown

PROVIDER NOTE (Liver): Mild-to-moderate LFT elevation is common in active AUD and is NOT a contraindication to naltrexone 50mg. The FDA black box warning is based on doses of 300mg/day -- not the 50mg therapeutic dose. Withhold prescribing only if: known cirrhosis, acute hepatitis, liver failure, or confirmed LFTs greater than 3-5x ULN. Continued heavy drinking causes far more liver damage than naltrexone 50mg. Order LFTs if uncertain.

24. Are you currently pregnant or breastfeeding?

- ☐ Yes -- currently pregnant
- ☐ Yes -- currently breastfeeding
- ☐ I am planning to become pregnant in the next 12 months
- ☐ No / Not applicable

PROVIDER NOTE (Pregnancy): Naltrexone is FDA Pregnancy Category C -- not an absolute contraindication, but requires individualized risk-benefit discussion with the patient. Untreated severe AUD in pregnancy carries significant fetal and maternal risks (fetal alcohol syndrome, preterm birth, placental abruption) that may outweigh naltrexone's unknown fetal risk. Naltrexone passes into breast milk -- generally avoid while breastfeeding. For all pregnant, breastfeeding, or potentially pregnant patients, consult with or refer to an OB/addiction medicine specialist before prescribing.

25. Do you have any known allergies to medications?

- ☐ Yes -- including to naltrexone or similar medications
- ☐ Yes -- other medications only
- ☐ No known drug allergies

SECTION 5 — Medical & Psychiatric History

26. Do you have a history of any of the following? (Select all that apply)

- ☐ Depression or anxiety
- ☐ Bipolar disorder
- ☐ PTSD or trauma history
- ☐ Kidney disease or chronic kidney condition
- ☐ Chronic pain requiring opioid medication
- ☐ Other significant medical conditions
- ☐ None of the above

27. Are you currently taking any prescription medications?

- ☐ Yes ☐ No

If yes, please list: _____

28. Have you tried any of the following for alcohol use before? (Select all that apply)

- ☐ Naltrexone (oral pill)
- ☐ Vivitrol (naltrexone injection)
- ☐ Acamprosate (Campral)
- ☐ Disulfiram (Antabuse)
- ☐ Individual therapy or counseling
- ☐ AA / 12-step program
- ☐ Inpatient or residential rehab
- ☐ None -- this is my first time seeking treatment

29. Are you currently seeing a therapist, counselor, or psychiatrist?

- ☐ Yes ☐ No

SECTION 6 — Treatment Goals & Readiness

30. What is your primary treatment goal?

- ☐ Stop drinking completely (full abstinence)
- ☐ Significantly reduce how much I drink (harm reduction / Sinclair Method)
- ☐ I am not sure yet -- I want guidance from my provider

31. How motivated are you to change your drinking right now?

- ☐ Very motivated -- I am ready to start
- ☐ Somewhat motivated -- I have some hesitation
- ☐ Unsure -- I am still exploring my options

32. Do you have a support system at home (family, friends, sponsor, or therapist)?

- ☐ Yes -- strong support
- ☐ Somewhat -- limited support
- ☐ No -- I am managing this on my own

SECTION 7 — Demographics

33. Biological sex (for clinical dosing reference):

- ☐ Male
- ☐ Female
- ☐ Prefer to self-describe: _____

34. Age: _____

PROVIDER DECISION SUMMARY

Clinical Signal	Provider Action
2+ DSM-5 criteria (Q1-11)	AUD diagnosis supported -- proceed with treatment planning
Q20 or Q21 = Yes (opioid use)	ABSOLUTE CONTRAINDICATION -- do not prescribe; patient must be opioid-free 7-10 days minimum (longer for methadone)
Q16, 17, or 18 = Yes (withdrawal history)	Escalate care -- evaluate for supervised detox before starting naltrexone
Q22-23: Cirrhosis / liver failure / LFTs >3-5x ULN	Withhold naltrexone -- further hepatic evaluation required
Q22-23: Mild or moderate LFT elevation only	May proceed -- monitor LFTs; risk of continued drinking far exceeds naltrexone 50mg hepatic risk
Q24 = Yes (pregnant or breastfeeding)	Avoid naltrexone -- consult OB/addiction specialist
Q30 = Harm reduction goal	Discuss Sinclair Method -- take naltrexone 1-2 hours before drinking

Q29 = No current therapist

Recommend adjunct behavioral support per ABAM guidelines

This intake form is intended for use by licensed healthcare providers and does not replace clinical judgment.
References: DSM-5 Diagnostic Criteria for Alcohol Use Disorder (APA, 2013) | American Board of Addiction Medicine (ABAM) Standards of Care | FDA Prescribing Information for Naltrexone HCl 50mg.